

● RED · ACT TODAY

FIND YOUR SITUATION · PATHWAY 1

YOUR SITUATION

"Mom had a stroke — she's in the hospital and they say she needs a skilled nursing facility."

THE ONE THING THAT PROTECTS EVERYTHING ELSE

Before anything else, ask the hospital — today, and get it in writing: "**Is my mother admitted as an inpatient, or under observation?**" Medicare only covers the SNF stay after a **3-day inpatient** hospital stay — observation days and the discharge day do not count. If she is on observation status, ask the doctor whether inpatient admission is appropriate. (Many Medicare Advantage plans waive this rule — call the plan and ask.) → *Chapters 10 and 13*

Your First Five Moves

1. **Confirm inpatient status** (above) and write down who told you, and when.
2. **Meet the discharge planner** — ask for a list of SNFs with open beds, then check each one on **medicare.gov/care-compare** before you agree. You may say: "We would like to understand our choices." → *Chapter 10*
3. **Get the medication list** and the names of her hospital doctors before the transfer — hand-offs are where medication errors happen. → *Chapter 25*
4. **Know the money clock:** with a qualifying stay, Medicare pays days 1–20 in full, days 21–100 with a daily coinsurance, and nothing after 100. Rehab rarely runs that long — but know the numbers before the bills arrive. → *Chapters 13–15*
5. **Attend the SNF care conference** (ask for one in week one). Ask: "What are the therapy goals, and what does discharge planning look like from here?"

THE TRAPDOOR TO WATCH

If the SNF says therapy will end because she has "plateaued" or is "not making progress" — **that is not the rule**. Medicare covers skilled care to maintain function too (the *Jimmo* standard). You can appeal the cut-off notice through your Medicare QIO — the fast appeal is free and often works. → *Chapter 10 has the script.*

While She Recovers — Two Quiet Tasks

☐ Learn the stroke warning signs for next time — **BE-FAST**: balance, eyes, face drooping, arm weakness

- Learn the stroke warning signs for next time — **BEFAST**: balance, eyes, face drooping, arm weakness, speech trouble → note the time, call 911. → *Chapter 2*
- If her paperwork is not done and she can still make decisions, this is the week: Healthcare Power of Attorney and advance directive. → *Chapters 20–21*

PATHWAY TAKEAWAY

Confirm **inpatient** status today, pick the SNF from real inspection data, and never accept "plateaued" as the end of therapy without an appeal.

YOUR SITUATION

"Dad has been declining — he can't take care of himself anymore."

FIRST, RULE OUT THE REVERSIBLE

If the change is **sudden** — days, not months — treat it as medical until proven otherwise: infections, dehydration, and medications cause confusion and weakness that can be reversed. Call his doctor today and describe what changed, with dates. → *Chapters 1–3*

Your First Five Moves

1. **Write the change log:** what he can no longer do safely — bathing, meals, medications, stairs, driving — with examples and dates. Facts move doctors. → *Chapter 1*
2. **Book a full evaluation** with his doctor. Bring the log and ask: "Is any of this reversible, and does he meet criteria for a home health evaluation?" → *Chapter 6*
3. **Match help to the real gaps** — not the brochure: daily hands-on help is private duty or family (Ch. 7); therapy and nursing at home is home health (Ch. 6); rising supervision needs point to assisted living or memory care (Ch. 8–9).
4. **Get the documents signed while he can:** Healthcare and Financial Power of Attorney and an advance directive require capacity — waiting can cost the family a guardianship case later. → *Chapters 20–22*
5. **Call the family meeting** before the crisis picks the date for you — who does what, who pays what, who decides what. → *Chapter 5*

THE TRAPDOOR TO WATCH

Medicare does not pay for long-term custodial care — not at home, not in assisted living, not in a nursing home. If his money may run short, start the Medicaid conversation early: the **five-year look-back** punishes money moved late, and an elder-law attorney before any transfer is cheaper than one after. → *Chapters 14–15*

And One Thing for You

- ☐ If you are the one absorbing the decline, read the burnout chapter before you think you need it — the scored self-check takes five minutes, and asking for help is planning, not failure. → *Chapter 4*

Rule out the reversible first, match help to the actual gaps, sign the documents while he can, and start the money conversation before the crisis starts it for you.

YOUR SITUATION

"I lost Mom years ago. Now Dad is declining — how do I protect his assets and make sure he gets care?"

THE HONEST ANSWER FIRST

"Protect all the assets" and "qualify for help paying for care" pull against each other, and the law knows it: Medicaid looks back **five years** at money and property moved for less than value, and late transfers create penalty months when he needs care most. The single highest-value move on this page: **see an elder-law attorney before anyone moves a dollar** — the consult costs a fraction of one look-back mistake. → *Chapters 14–15*

Your First Five Moves

1. **Documents while he has capacity:** durable Financial Power of Attorney, Healthcare Power of Attorney, advance directive. Without them, you may need a guardianship case just to help him. → *Chapters 20–22*
2. **Build the inventory:** accounts, deed, income, insurance policies — including any **long-term care insurance** he may have forgotten he owns. One folder. → *Chapters 16, 19*
3. **Elder-law consult** (look for CELA certification or a NAELA member): what is exempt, what spend-down would look like, whether any planning still makes sense at his timeline — *before* any transfer, "adding names" to accounts, or signing the house over.
4. **Match care to today's gaps** — home health evaluation, private duty hours, adult day, or a residential move. → *Chapters 6–9*
5. **If he ever served:** ask about VA pension with Aid and Attendance — and note the VA has its own **36-month look-back**. → *Chapter 18*

THE TRAPDOOR TO WATCH

The neighbor's advice — "**just put the house in your name**" — is how families create Medicaid penalty periods and surprise tax bills at the same time. Widowed parents managing money alone are also prime targets for exploitation: set up account alerts, and treat any new "friend" with financial interest as a red flag. → *Chapter 26*

Documents first, inventory second, elder-law attorney before any transfer — and match the care to the gaps he has now, not the crisis you fear later.

"Mom is deciding between Original Medicare and Medicare Advantage."

THE TRADE IN ONE PARAGRAPH

Original Medicare (usually with a Medigap supplement and a Part D drug plan) costs more per month, but nearly any doctor or hospital that takes Medicare, with few prior authorizations. **Medicare Advantage** replaces that with one private plan — lower premium, often dental/vision extras — in exchange for networks, prior authorization, and copays that stack up in a bad year. Neither is "wrong." One question decides most cases: *which would she rather have in the year she gets really sick?* → Chapter 13

The Five Questions to Answer Before Choosing

1. Are **her doctors and preferred hospital** in the Advantage plan's network — and what happens if they leave it?
2. Are **her medications** on the plan's formulary, and at what tier? (Run them through the Medicare Plan Finder at [medicare.gov](https://www.medicare.gov).)
3. Can she **afford the Medigap premium now**? Predictable premiums versus unpredictable copays is the real math.
4. Does she **travel or split the year** between states? Networks travel poorly; Original Medicare goes everywhere.
5. How would she feel about **prior authorization** for rehab or home health during a serious illness?

THE TRAPDOOR TO WATCH

The choice is **stickier than it looks**. Leaving Advantage later usually means buying Medigap *with medical underwriting* — in most states the insurer can charge more or say no because of her health, exactly when she most wants back in. Ask SHIP about guaranteed-issue windows and trial rights *before* choosing, not after.

Free, Unbiased Help

- ☐ **SHIP** — free, unbiased Medicare counseling: **1-877-839-2675** · shiphelp.org (they will sit with her and run her exact doctors and drugs)

- ☐ **Open Enrollment** runs October 15 – December 7 each year; plans change their networks and formularies annually — recheck even if she is happy

PATHWAY TAKEAWAY

Run her actual doctors and drugs through the plan, price the bad year and not just the premium, and understand the Medigap underwriting trap before — not after — she chooses.